Emergency Admission Policy

SHIFA General Hospital (Fictional)

DEMO / SYNTHETIC HOSPITAL DATA — NOT FOR REAL CLINICAL USE

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1. Purpose and Scope

This policy explains how patients who arrive at the SHIFA General Hospital Emergency Department (ED) are received, registered, assessed and, when needed, admitted. It applies to all ED doctors, nurses, registration clerks, security staff and the Bed Management team.

The ED is open 24 hours a day, 7 days a week. The ED direct extension is 1000.

2. Guiding Principles

- Assessment and stabilisation come first. Paperwork, identity documents, deposits and insurance approvals must never delay life-saving treatment.
- No patient is refused initial assessment because of inability to pay, a missing CNIC or a missing insurance approval.
- Medico-legal or police formalities never delay treatment.

3. Emergency Admission Procedure

Step 1 - Arrival and triage: Every patient is seen by the triage nurse within 5 minutes of arrival and given a triage category from 1 to 5 as described in EM-02 Triage Guidelines. Ambulance patients are received at the ambulance bay by an ED nurse. Category 1 patients go directly to the resuscitation bay.

Step 2 - Quick registration: The ED registration clerk creates an emergency MRN using minimum data: name (or an alias such as Unknown-Male-01 if the patient cannot be identified), approximate age, sex, mode of arrival and a relative's contact number. Full registration under ADM-01 is completed within 24 hours by the Admissions Desk.

Step 3 - Doctor assessment: The ED doctor assesses the patient within the time target of the triage category (see EM-02).

Step 4 - Consent: Written consent for treatment is taken from the patient or guardian. If the patient cannot give consent and no relative is present, two physicians (one of them a senior doctor) document an emergency consent in the chart before life-saving treatment begins.

Step 5 - Disposition decision: The ED physician decides between (a) discharge with written instructions, (b) ED observation for up to 6 hours, or (c) inpatient admission.

Step 6 - Admission request: For admission, the ED physician contacts the on-call specialty consultant, completes the Admission Order and Admission Note, and sends a bed request to Bed Management (extension 1150). The target is to allocate a bed within 60 minutes of the admission decision. If no ward bed is free, the patient stays in the ED under ED nursing care and Bed Management escalates to the Duty Manager after 60 minutes.

Step 7 - Identification and handover: An ID wristband is applied before the patient leaves the ED (see PS-01). The ED nurse gives an SBAR handover to the ward nurse (see NR-01).

Step 8 - Financial counselling: The billing counsellor meets the family only after the patient is stabilised. The admission deposit may be deferred for up to 24 hours after stabilisation (see BF-01).

4. Medico-Legal Cases (MLC)

Road traffic accidents, assault, firearm injuries, burns, suspected poisoning and suspected abuse are flagged as MLC by the triage nurse or treating doctor. The Medico-Legal Officer and hospital security are informed. The patient is treated first; police formalities are completed in parallel. MLC papers are kept in a separate confidential register.

5. Transfer to Another Hospital

If the required service is not available at SHIFA, the treating consultant decides on transfer only after stabilisation. A physician-to-physician handover is mandatory, the patient travels in an ambulance accompanied by a nurse, and a Transfer Form is completed. The patient or guardian is informed and gives consent unless the delay would be unsafe.

6. Mass Casualty Incident

In a mass casualty incident the ED in-charge or Duty Manager activates Code Red by calling extension 1999. Elective ED activity pauses and the hospital disaster plan takes over.